

Prayers of Care Inc

Strategic Plan — three-year

CARF Community Integration (BH) Binder — document STRAT-01

DRAFT — no approval date, no effective date. Do not date until signed.

Nicky Ingram, Chief Executive Officer / Owner
Three-year plan — inputs in section 3 before goals in section 5
CARF 1.C Strategic Planning (p.49)
Status: DRAFT — not yet adopted, not yet effective
Prepared with Successful Solutions

Document ID	STRAT-01
Version	Draft 1
Status	Draft → Approved → Trained → In use → Tested → Monitoring *(circle the current state)*
Plan period	_____ to _____ *(three years from the effective date)*
Owner	Nicky Ingram, Chief Executive Officer / Owner
Approval date	_____
Effective date	_____
Review interval	Progress reviewed quarterly; whole plan reviewed at least annually
Next review due	_____
Legal entity	Prayers of Care Inc
NPI	1043895022
CARF area served	1.C Strategic Planning (p.49) — with 1.D (p.53), 1.F (p.57), 1.G (p.61), 1.L (p.95), 1.M (p.107) feeding it
Program focus	Community Integration (3.C, p.185), delivered through peer support
Consultant	Successful Solutions (Tom Garner) — consultant only; not an officer or employee of Prayers of Care Inc

Confirmed: owner-operated, no board at this time · owner plus 4–10 staff · already serving people

· **no medication management offered** · no faith-based activity offered.

Two corrections carried into this draft. The draft named *Tom Garner, Owner / ED* as responsible party and as owner of Goals 1 and 6. Tom Garner is the **consultant**. The responsible party for every goal in this plan is **Nicky Ingram, Chief Executive Officer / Owner** — a surveyor who finds the consultant listed as the accountable officer will read it as the agency outsourcing its own accountability, which is a finding on its own. The draft also stated the agency opened in **November 2025**. That date is not in the confirmed record, so it is blank below. Fill it in — it is not a small blank, it drives every "how long have you been doing this" answer at survey.

Purpose

Set the direction of Prayers of Care Inc for three years, and show that the direction came from evidence rather than from opinion.

CARF does not ask whether an agency has ambitions. It asks to see the plan, the **inputs the plan was built from**, the review that happened afterwards, and what changed as a result. A plan with goals but no documented inputs is the single most common way 1.C is missed, and it is why section 3 of this document exists.

How this plan is meant to be used

1. **Fill in section 3 first, not section 5.** Goals written before the inputs are collected are guesses. Goals written after them are strategy.
2. **Nothing in section 5 is adopted until it has a number and a name.** "Improve quality" is not an objective. "80% of plans show a community activity goal, checked monthly by the Program Supervisor" is.
3. **Review it quarterly in the leadership meeting** (MTG-01, agenda item 7) and record the review in those minutes. Review the whole plan annually and record what changed in section 8.
4. **Adopting it is a Decision Log entry.** Prayers of Care Inc has no board, so adoption and every later change goes into the LEAD-01 section G Decision Log, not just into this file.
5. **Tell people about it.** Section 9. A plan nobody has seen is not a plan.

1. Who we are — snapshot

Item	Detail
Legal entity	Prayers of Care Inc
NPI	1043895022
Date the agency began delivering services	`[FILL IN — month and year]`
CARF program sought	Community Integration (BH), 3.C
Delivery model	Peer support

Item	Detail
Counties actually served	`[FILL IN]`
Sites operated	`[FILL IN]`
Current number of people served	`[FILL IN]`
Maximum the agency can serve at today's staffing	`[FILL IN]` — and the reasoning goes in the staffing plan, not here
Workforce	Owner plus 4–10 people. Roster: `[FILL IN]`
Payers	`[FILL IN]`
EHR / record system	`[FILL IN]`

Capacity is not a guess. A surveyor asks "how many people can one peer specialist carry, and how did you decide that?" The number above and the caseload cap in JD-02 must be the same number, and there must be a reason behind it — hours available, travel time in the counties served, documentation time. Two different numbers in two documents is a finding.

2. Mission, vision and values

Not repeated here. **The authoritative statement is MVV-01.** Strategy that contradicts the mission is a problem a surveyor will spot, so read MVV-01 before writing goals, and if a goal requires changing the mission, change MVV-01 deliberately and log the decision.

3. Inputs this plan was built from — complete this before section 5

Record where each input came from and when it was gathered. Leave a row blank only if the input genuinely does not exist yet, and then write **"not yet available"** and say when it will be — a documented gap is accepted, a silent one is not.

Input	Where it comes from	Date gathered	What it showed
What the people served say they need	Satisfaction and input survey, analysed per 1.D		
What other stakeholders say	Stakeholder survey — referral sources, payers, family, community partners		
Community need in the counties served	County and state behavioral health data		
Regulatory and payer environment	NC rules, payer bulletins, contract terms		
Financial position and projection	Budget vs actual, cash position		
Technology capability and need	Record system, security, remote access		
Workforce supply, turnover and credentialing	Vacancy days, turnover, credential renewals		
Risk profile	Risk register — including lone working in the community		
Performance results	Measures in the Evidence and Data Workbook		
Who else provides this service locally	`[FILL IN — source]`		

Where these are recorded: the Evidence and Data Workbook holds the numbers; the survey and analysis documents hold the input; this table is the index that ties them to the plan.

4. Strengths, challenges, opportunities and threats

Write these from section 3, not from memory. Each line should be traceable to a row above.

	Item	Which input row it came from
Strength	Peer-delivered service by people with lived experience	
Strength	One clearly defined program rather than several half-built ones	
Strength	Owner directly involved in day-to-day operation	
Strength	`[FILL IN]`	
Challenge	New agency — limited written history to show a surveyor	
Challenge	Small team: one absence removes a whole function	
Challenge	Documentation load on peer staff	
Challenge	Emergency drills and reviews have not yet run a full cycle	
Challenge	`[FILL IN]`	
Opportunity	Referral relationships with `[FILL IN]`	

	Item	Which input row it came from
Opportunity	Consumer-Run specialty designation (5.D, p.318) if the agency chooses to pursue it	
Opportunity	`[FILL IN]`	
Threat	Dependence on a single payer or contract	
Threat	Peer credential lapses stopping billing	
Threat	Key-person risk — one person holding access, knowledge or signing authority	
Threat	`[FILL IN]`	

5. Strategic goals

Every goal carries a **measurable objective, a responsible position, a target, and the resources it needs**. Owners are named by **position**; the person holding each position is in the ORG-01

function assignment table, which stays the single source of truth.

Years are counted from the effective date on the cover — Year 1 is the first twelve months — so this plan does not go stale the moment a date slips.

Goal 1 — Accreditation readiness

Outcome	A survey-ready Community Integration binder and operations that have actually been practised, leading to CARF accreditation
Year 1	Binder foundation complete; remaining ASPIRE areas started; every policy adopted, trained and in use — not merely written
Year 2	Legal, financial, risk, technology, rights, accessibility and quality packs complete; mock survey run and findings closed
Year 3	Survey applied for and completed per CARF's own timing guidance; conformance sustained after the award
Responsible	Chief Executive Officer / Owner
Measures	% of the master checklist complete · drills and training current · every document past Draft in the six-state status · mock survey findings closed
Resources	Consultant time, staff time, manual purchase, CARF fees, `[FILL IN]`

Say nothing that is not yet true. Until CARF awards accreditation, the agency does not describe itself as accredited, CARF-approved, or in any wording a reader could take that way — on the website, in marketing, on proposals, or in a payer application. "Preparing for CARF accreditation" is accurate and is enough. A false claim is a serious problem with CARF and with payers, and it is entirely avoidable.

Goal 2 — Quality peer-delivered Community Integration

Outcome	Every person served has a current person-centred plan, in their own words, with community participation goals and the natural supports around them recorded
Year 1	Plan and note templates in use for every admission; orientation completed and documented for 100% of new admissions
Year 2	`[FILL IN]` % of plans show a community activity goal with natural supports named; plan reviews completed on time
Year 3	Outcomes summarised and reported back to persons served and staff in plain language
Responsible	Program Supervisor
Measures	Monthly record review sample · plan reviews completed by their due date · satisfaction of persons served
Resources	Supervisor time for record review, `[FILL IN]`

Use one name for the plan. The draft called it an *Individual Community Participation Plan*. Whatever it is called, the name in this plan, on the form itself, in the policy and in what staff say out loud has to match. Person-centred planning is 2.C (p.141); the form is what a surveyor pulls from the record.

Goal 3 — Workforce stability and peer competency

Outcome	A trained, supervised peer team working within clear boundaries
Year 1	Orientation and competency assessment delivered and documented for every staff member; supervision logged from the first month
Year 2	Retention approach in place; advanced training topics delivered; credential renewals tracked ahead of expiry
Year 3	A trained backup exists for every key function — nothing depends on one person
Responsible	Chief Executive Officer / Owner, with the Program Supervisor
Measures	Training completion % · supervision delivered vs required · vacancy days · credentials verified before hire, 100%
Resources	Training budget `[FILL IN]`, supervision time `[FILL IN]`

Competency is not attendance. Signing a training register is not evidence that someone can do the work. Competency is observed and recorded — 1.I (p.80). This is the difference a surveyor tests when they pull three personnel files.

Goal 4 — Community partnerships and referral network

Outcome	A reliable referral and resource network, so community integration means something specific rather than a word in a policy
Year 1	Top ten partners identified and listed; stakeholder survey fielded and analysed
Year 2	Written referral pathways or MOUs with `[FILL IN]` partners
Year 3	Joint community activity or resource event where it serves the people served
Responsible	`[FILL IN — position, from ORG-01]`
Measures	Number of active referral sources · partner survey ratings · community resource directory reviewed and current
Resources	`[FILL IN]`

Goal 5 — Safety and risk discipline

Outcome	Drills that actually happen, and incidents that lead to a change
Year 1	Drill schedule run and documented for every emergency type; incident reporting in use from day one
Year 2	Incident and drill trends reviewed quarterly, with at least one documented change made because of them
Year 3	Drill and review schedule sustained at target with no gaps
Responsible	Health and Safety function holder — see ORG-01
Measures	Drills completed vs scheduled · time to close an incident · lone-working checks for staff in the community
Resources	`[FILL IN]`

Community-based delivery is the risk that matters here. Peer specialists work in people's homes and in the community, often alone. Drills for a building are the easy half; the check-in procedure for a staff member at a home visit is the half a surveyor asks a peer specialist to describe from memory — 1.H (p.65).

Goal 6 — Sustainable operations

Outcome	A budget that actually supports the staffing and compliance the other five goals require
Year 1	Written operating budget; budget compared to actual on a stated cycle; billing integrity basics in writing
Year 2	Insurance and reserve position reviewed: `[FILL IN]`; external financial review obtained
Year 3	Capacity matched to demand without quality dropping
Responsible	Chief Executive Officer / Owner
Measures	Budget vs actual variance · claim denial rate · internal control checks — who approves, who reconciles
Resources	Accountant `[FILL IN]`, `[FILL IN]`

Separation of duties in a one-owner agency. Financial management (1.F, p.57) expects that the person who approves a payment is not the only person who reviews it. Where the Owner is both, name the external accountant or bookkeeper who provides the second look, and record it as a known risk in the risk register. Saying nothing is what gets written up.

6. Resources this plan requires

Resource	Amount or level needed	Where it comes from	Confirmed?
Peer support FTEs	`[FILL IN]`		
Supervisor time per week	`[FILL IN]`		
Training budget	`[FILL IN]`		
CARF fees and standards manual	`[FILL IN]`		
Record system and secure file storage	`[FILL IN]`		
Consultant support	`[FILL IN]`		
Insurance	`[FILL IN]`		

A goal with no resource line behind it is the first one to quietly fail. If a resource is not available, the honest move is to change the goal, not to leave both in place.

7. How progress is reviewed

Review	Who	When	Recorded in
Quarterly progress against every goal	Leadership	Quarterly	MTG-01 leadership minutes, agenda item 7
Annual review of the whole plan	Chief Executive Officer / Owner	At least annually	Section 8 below, plus the LEAD-01 Decision Log
Any change to a goal, a target or the plan period	Chief Executive Officer / Owner	When it happens	LEAD-01 section G Decision Log

The annual review records objectives **not met** as well as met, and what will change because of it. A review that reports only successes is not a review, and a surveyor reads it that way.

8. Annual review record

Review due	Completed (date)	Reviewed by	Objectives met	Objectives not met, and what changes	New version
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9. Communicating this plan

Audience	What they get	How	Date done
Staff	Plain-language summary, and their part in it	`[FILL IN]`	
Persons served	Plain-language summary of what the agency is working towards	`[FILL IN]`	
Partners and payers, where appropriate	Summary	`[FILL IN]`	

Record that the distribution happened. Distribution nobody recorded did not happen, as far as a surveyor is concerned.

10. Adoption

This is a **working draft until signed**, and adoption is recorded in the Decision Log.

Role	Name	Signature	Date
Chief Executive Officer / Owner	Nicky Ingram	_____	_____
Program Supervisor	`[FILL IN]`	_____	_____

Step	Done
Section 3 inputs gathered and recorded	☐
Goals given numbers, positions and resources	☐
Adopted and signed above	☐
Copied into the LEAD-01 Decision Log	☐
Shared with staff and persons served (section 9)	☐
Quarterly review scheduled into the leadership meeting	☐

What a surveyor will ask

They ask	They are testing	What has to be true
"Show me the strategic plan."	1.C exists at all	This document, signed and dated
"What data went into it?"	The real test of 1.C	Section 3, with sources and dates — not opinions
"Show me the review, and what changed as a result."	Whether the plan is live	Section 8, plus the quarterly entries in the leadership minutes
"Who is accountable for this goal?"	Named accountability	A position in section 5, and the same name in ORG-01
"How did the people you serve influence this plan?"	1.C together with 1.D	The input row in section 3, traced back to the survey analysis
"How many people can you serve, and how do you know?"	2.A staffing sufficiency	The capacity figure in section 1, matching JD-02's caseload cap, with reasoning
"Are you telling anyone you're accredited?"	Truthful representation	The website and every document say *preparing for* accreditation, nothing more
Asked of a peer specialist : "what is the agency trying to achieve this year?"	Whether the plan reached the staff	Their answer matches section 9's summary

Still needed before STRAT-01 can be adopted

- **The date the agency began delivering services** — removed from the draft because it is not in the confirmed record. It drives the evidence clock and the survey timing
 - Counties served, sites, current numbers served, and the defensible capacity figure
 - Payers, and whether any single payer is a concentration risk worth naming in section 4
 - Program Supervisor's name, and the position holder for Goal 4
 - The Health and Safety function holder (ORG-01)
 - Budget figures, training budget, insurance, and the external accountant who provides the second look on finances
 - Whether the Consumer-Run specialty designation (5.D, p.318) is being pursued — it changes Goal 1
 - Target survey month, once CARF's timing guidance is confirmed with the resource specialist
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